

LONG COVID: ROUTINE CARE

A patient is considered to have 'Long COVID' if s/he has ongoing symptoms for 2 or more months following acute COVID-19 infection (usually 3 months from the onset of COVID-19 symptoms).

Assess the patient with Long COVID

Assess	When to assess	Note
Symptoms	Every visit	- Ask about symptom/s: specifically ask about difficulty sleeping and ongoing pain. Manage as on symptom pages. - If patient was hospitalised for COVID-19 and breathlessness lasts > 6 weeks after discharge, refer to physiotherapist, if available, for assessment/home programme. - If persistent dry cough ≥ 8 weeks, consider referral to a speech and language therapist, if available. - If symptoms still present and troubling after 12 weeks, or uncommon (like palpitations, skin rash), refer/discuss with doctor/specialist.
TB	Every visit	Follow up TB sputum results. If no TB sputums sent during 'Long COVID' work-up, send 1 sputum sample for TB NAAT at this visit ➔ 92.
Daily activities	Every visit	If patient not able to cope with activities of daily living (like bathing, dressing, grooming, homemaking), consider referral to available rehab team member.
Chronic conditions	Every visit	If patient has chronic condition, check control and give routine care. Check that routine bloods have been done.
Mental health	Every visit	If stress, anxiety or low mood, assess and manage further ➔ 86.
Family planning	At diagnosis	If patient had severe COVID-19 and is on combined oral contraceptive (COC), doctor to discuss risks of thrombosis and consider switch to progestogen-only pill, copper IUD or subdermal implant. Assess family planning needs ➔ 154.
Carer/family	Every visit	Ask how carer/family is coping.
CVD risk	At diagnosis	Assess CVD risk at diagnosis ➔ 127.
Weight	Every visit	If weight loss, assess further ➔ 23.
Chest x-ray	If cough/breathlessness ≥ 12 weeks	If chest x-ray abnormal, refer/discuss.
Thyroid	Tiredness ≥ 12 weeks	Check TSH. If abnormal, refer to doctor.

Advise the patient with Long COVID

- Reassure that many people with COVID-19 have ongoing symptoms, even in mild cases. Explain that, normally, symptoms slowly resolve without specific treatment.
- Advise that symptoms may fluctuate and to expect good days and bad days. Advise to rest and pace activity. Set achievable targets and gradually increase activity according to symptom severity.
- Advise to look after general health: eat a healthy diet, get enough sleep, limit alcohol and caffeine and avoid illicit drugs.
- Extend sick leave as needed. Suggest patient speaks to employer about options to return to work more slowly. If unemployed, refer to SASSA to apply for COVID-19 Social Relief of Distress Grant.
- If needed, discuss what can be done to support carer/s and family. Identify local resources, social worker, counsellor, NGO, community action networks. Refer to occupational therapy if available.

Treat the patient with Long COVID

- Treat pain with **paracetamol** 1g 4-6 hourly (up to 4g in 24 hours) or **ibuprofen** 400mg 8 hourly with food for up to 2 weeks. Review need for pain medications after 2 weeks.
- Chronic overuse may cause headaches: if using painkillers > 2 days/week for ≥ 3 months, advise to reduce or stop pain medication.
- Encourage patient to get vaccinated unless unwell and being actively investigated, then discuss with specialist.
- Help patient to manage ongoing symptoms of tiredness, breathlessness and cough ➔ 122. Avoid prescribing inhalers used for asthma to treat breathlessness unless patient known with asthma.

Review the patient with Long COVID

- If TB sputums sent at this visit: review in 2 days, otherwise review 2-4 weekly as needed. Expect gradual improvement.
- If no gradual improvement, refer/discuss. Advise to return urgently if breathlessness worsens, new or worsening confusion or unable to wake patient, chest pain or pressure that won't go away, new sudden weakness or numbness in face, leg or arm: refer.